Ethics Risk Assessment Template

IMMU-201 | Phase II Immunotherapy Trial

Synthetic training data - educational use only.

Instructions

Read the case study below together with the IMMU-201 Protocol Summary. One ethical risk has been identified and documented as a model. Use the same format and level of specificity to identify and document at least two additional risks.

- Name the ethical risk precisely.
- Identify the participant or population affected.
- Describe the protocol or scenario evidence that creates the concern.
- Assign a severity level and explain your reasoning.
- Recommend a concrete protective action or escalation pathway.
- Do not copy the model risk into the remaining rows.

Case Study Scenario

The IMMU-201 site is preparing to enroll participants in the randomized, double-blind, placebo-controlled Phase II immunotherapy study. Adults with moderate-to-severe rheumatoid arthritis may be eligible if they remain symptomatic despite stable standard therapy.

During a pre-screening discussion, a potential participant says, “I really need this new treatment to work because my current medication is not helping.” A staff member responds that the study drug is promising and says the participant will “probably feel much better once treatment starts.” The participant has not yet completed the informed consent discussion.

Another potential participant has difficulty reading the consent form and asks whether a family member can simply tell them where to sign because they do not want to lose the opportunity to join the study. The clinic is running behind schedule, and the consent discussion is shortened so the screening procedures can begin on time.

At a later study visit, a randomized participant reports fever, worsening cough, and fatigue after receiving study injections. The participant is concerned that reporting symptoms might cause removal from the study and initially asks the coordinator not to document them. The participant is receiving an investigational therapy that alters immune activity.

A participant who can become pregnant is uncomfortable discussing contraception and pregnancy testing in an open clinical work area where other patients and staff can hear the conversation. The study requires pregnancy-related precautions when applicable.

Several participants also report that the repeated visits, injections, blood draws, joint examinations, and questionnaires are burdensome, particularly when visits take longer than expected.

Risk Documentation Table

The first entry is modeled. Complete the remaining entries using evidence from the case study and Protocol Summary.

Risk	Population Affected	Evidence / Concern	Severity	Protective Action / Escalation
Therapeutic misconception / undue influence during recruitment and consent	Potential participants seeking better control of rheumatoid arthritis symptoms	A staff member tells a potential participant that the investigational treatment is promising and that they will probably feel better. This may create an unrealistic expectation of direct benefit and can undermine an informed, voluntary decision, particularly because the study includes randomization and possible placebo assignment.	High - the statement occurs before consent and may materially affect the participant's understanding of research participation and likelihood of benefit.	Stop and correct the misleading statement before consent proceeds. Ensure the consent discussion clearly explains investigational status, randomization, placebo assignment, foreseeable risks, alternatives, and that benefit is not guaranteed. Escalate the concern to the PI/site leadership and document/retrain as required by site procedures.
Risk 2: Inadequate informed consent: impaired comprehension and a rushed consent discussion	Potential participants who have difficulty reading or understanding the consent form (limited literacy, visual or cognitive impairment, or another condition that may affect decision-making),	A potential participant "has difficulty reading the consent form" and asks whether a family member can "simply tell them where to sign" because they do not want to lose the opportunity to join.	High - a signature obtained without understanding is not valid consent. Every later procedure, including screening blood draws and randomization to possible placebo, would be	Do not start screening procedures. Restart or reschedule the consent discussion with enough time. Read and explain the full form, confirm understanding

	and any participant consented under time pressure.	The clinic is behind schedule and "the consent discussion is shortened so the screening procedures can begin on time." The protocol requires written informed consent before any protocol-specific procedure and adequate time for questions and decision-making (Sections 5, 8, and 10).	performed on a person who has not validly agreed.	with teach-back, and use an impartial witness or other accommodation as the IRB-approved process requires. Refer any concern about capacity to the PI. If any procedure was done before valid consent, report it to the PI as a protocol deviation for sponsor and IRB reporting, and retrain staff.
Risk 3: Unreported and undocumented safety symptoms (possible infection) because the participant fears removal from the study	Randomized participants receiving blinded IMT-201 or placebo injections, especially any participant with new symptoms of infection.	A randomized participant reports "fever, worsening cough, and fatigue after receiving study injections," fears that reporting "might cause removal from the study," and "asks the coordinator not to document them." The case notes the therapy "alters immune activity." Protocol Section 9 (Risk 1) identifies increased or altered infection risk and requires "prompt adverse-event assessment and investigator escalation"; Section 10 requires AEs and	High - fever and worsening cough in a participant on an immune-altering investigational product may indicate a serious infection. Delay in assessment could cause serious physical harm, and omitting the event would also compromise the study's safety data.	Document the symptoms in source the same day and explain that the coordinator cannot leave them out. Notify the PI or sub-investigator immediately for medical assessment before any further injection. Record the adverse event; if serious criteria are met, report as an SAE within protocol timelines. Reassure the participant that reporting is for their safety and that they keep the right to continue

		SAEs to be assessed, documented, and reported.		care or withdraw.
Risk 4: Loss of privacy and confidentiality during reproductive health discussions	Participants who can become pregnant and are subject to pregnancy testing and pregnancy-prevention requirements.	A participant who can become pregnant "is uncomfortable discussing contraception and pregnancy testing in an open clinical work area where other patients and staff can hear the conversation." Protocol Section 8 states pregnancy testing "involves sensitive reproductive information and should be handled privately," Section 9 (Risk 3) requires these measures to be "discussed privately and documented," and Section 10 requires participant confidentiality to be protected.	Moderate - there is no immediate physical harm, but sensitive personal information is exposed to people with no need to know it. A participant who feels overheard may also withhold information, which increases the risk of an undetected pregnancy and fetal exposure.	Stop the conversation and move to a private room before continuing. Make a private space a standing requirement for consent, pregnancy testing, contraception counseling, and questionnaires. Document the testing and discussion in source. Report the event to the PI and handle it under the site's privacy incident procedure if identifiable information was overheard.
Risk 5: Cumulative procedural burden affecting participant welfare, retention, and equitable participation	All enrolled participants over the 24-week treatment period, particularly those for whom long or frequent visits are hardest to manage (for example, because of pain and limited function from	Several participants report that "the repeated visits, injections, blood draws, joint examinations, and questionnaires are burdensome, particularly when visits take longer than expected." The protocol	Low to Moderate - each procedure carries minor, expected discomfort, but the burden accumulates. It is reported by several participants and may lead to missed visits or	Keep visits within protocol windows and avoid unnecessary repeat procedures. Sequence procedures to shorten visits and tell participants in advance how long each visit will

	active rheumatoid arthritis, work, or caregiving).	schedules eight visits (Screening, Baseline/Day 1, and Weeks 4, 8, 12, 16, 20, and 24), and Section 9 (Risk 4) recognizes that repeated procedures "may create discomfort, inconvenience, or fatigue."	withdrawal, and it falls most heavily on participants with the fewest resources.	take. Record burden concerns and report them to the PI. Remind participants that they may withdraw at any time without penalty. Escalate to the sponsor if burden is causing missed visits or withdrawals.

Detailed Analysis

Use this section to expand on the risks you entered in the table if additional explanation is needed.

Risk 2 Analysis
Risk type: Name the ethical concern.

Inadequate informed consent: impaired comprehension and a rushed consent discussion.

Who is affected: Identify the participant or population affected.

Potential participants who have difficulty reading or understanding the consent form (limited literacy, visual or cognitive impairment, or another condition that may affect decision-making), and any participant consented under time pressure.

Evidence: Cite the specific fact or behavior in the case study or Protocol Summary that supports your concern.

A potential participant "has difficulty reading the consent form" and asks whether a family member can "simply tell them where to sign" because they do not want to lose the opportunity to join. The clinic is behind schedule and "the consent discussion is shortened so the screening procedures can begin on time." The protocol requires written informed consent before any protocol-specific procedure and adequate time for questions and decision-making (Sections 5, 8, and 10).

Why it matters: Explain the potential effect on participant rights, safety, welfare, privacy, or voluntariness.

Consent protects the participant's right to decide. This participant cannot independently read the document that explains randomization, placebo assignment, infection risk, alternatives, and the right to withdraw, and is motivated by fear of losing a place in the study. Shortening the discussion removes the only remaining opportunity to understand. A family member can support the participant but cannot replace the participant's own understanding and decision. There is also an equity concern in both directions: the person must not be enrolled without understanding, and must not be turned away simply because they need the information delivered differently. Reading difficulty does not by itself mean a

person lacks capacity, but the site must find out why the difficulty exists and whether it affects the ability to understand and choose.

Recommended action: State the immediate protection, correction, or escalation you recommend.

Stop before any protocol-specific procedure. Give the participant a new appointment if the clinic cannot provide adequate time today, and reassure them that taking time will not cost them the opportunity. Conduct the full discussion orally in plain language, check understanding of the key points by asking the participant to explain them back, and follow the IRB-approved procedure for participants who cannot read (for example, an impartial witness present for the whole discussion). The PI or a delegated investigator should assess and document capacity if there is any doubt. Document the process in source. Escalate to the PI; report to the sponsor and IRB if procedures occurred before valid consent; retrain delegated consent staff and remove scheduling pressure from consent visits.

Risk 3 Analysis
Risk type: Name the ethical concern.

Unreported and undocumented safety symptoms (possible infection) because the participant fears removal from the study.

Who is affected: Identify the participant or population affected.

Randomized participants receiving blinded IMT-201 or placebo injections, especially any participant with new symptoms of infection.

Evidence: Cite the specific fact or behavior in the case study or Protocol Summary that supports your concern.

A randomized participant reports "fever, worsening cough, and fatigue after receiving study injections," fears that reporting "might cause removal from the study," and "asks the coordinator not to document them." The case notes the therapy "alters immune activity." Protocol Section 9 (Risk 1) identifies increased or altered infection risk and requires "prompt adverse-event assessment and investigator escalation"; Section 10 requires AEs and SAEs to be assessed, documented, and reported.

Why it matters: Explain the potential effect on participant rights, safety, welfare, privacy, or voluntariness.

The participant's immediate safety is at stake: the protocol excludes people with active serious infection precisely because IMT-201 may alter the body's response to infection. The request also shows a welfare and voluntariness problem, because the participant feels they must hide symptoms to keep access to a treatment they hope will help. If the symptoms are not documented, the investigator cannot protect this participant, and the safety and tolerability objective is undermined for every future patient who might receive IMT-201.

Recommended action: State the immediate protection, correction, or escalation you recommend.

Respond with empathy, but be clear that all reported symptoms must be documented and assessed. Record onset, severity, and the participant's own words in source, and escalate to the PI or sub-investigator at once for evaluation and a decision on whether dosing should be held. Complete AE documentation, including seriousness, severity, and relationship as assessed by the investigator, and follow SAE timelines if criteria are met. Emergency unblinding should be requested only if medically

necessary. Explain that a safety assessment does not automatically end participation and that any decision will be made for their protection. Remind all participants at each visit to report symptoms promptly between visits.

Risk 4 Analysis
Risk type: Name the ethical concern.

Loss of privacy and confidentiality during reproductive health discussions.

Who is affected: Identify the participant or population affected.

Participants who can become pregnant and are subject to pregnancy testing and pregnancy-prevention requirements.

Evidence: Cite the specific fact or behavior in the case study or Protocol Summary that supports your concern.

A participant who can become pregnant "is uncomfortable discussing contraception and pregnancy testing in an open clinical work area where other patients and staff can hear the conversation." Protocol Section 8 states pregnancy testing "involves sensitive reproductive information and should be handled privately," Section 9 (Risk 3) requires these measures to be "discussed privately and documented," and Section 10 requires participant confidentiality to be protected.

Why it matters: Explain the potential effect on participant rights, safety, welfare, privacy, or voluntariness.

Confidentiality is a participant right and a GCP requirement, and reproductive information is among the most sensitive data the study collects. The setting also affects safety: the effects of IMT-201 on pregnancy are not established and pregnancy is an exclusion criterion, so the protections only work if the participant feels able to speak honestly about contraception and possible pregnancy. Participants who can become pregnant carry this additional requirement, so failing to provide privacy places an unequal burden on them.

Recommended action: State the immediate protection, correction, or escalation you recommend.

Move the discussion to a private room immediately and acknowledge the participant's discomfort. Build privacy into the visit workflow by reserving a room for sensitive procedures and limiting those present to delegated staff who need to be there. Confirm that pregnancy test results and counseling are recorded only in access-controlled source records. Notify the PI, follow the institution's privacy incident process if a disclosure occurred, and brief all study staff on where sensitive conversations may take place.

Risk 5 Analysis
Risk type: Name the ethical concern.

Cumulative procedural burden affecting participant welfare, retention, and equitable participation.

Who is affected: Identify the participant or population affected.

All enrolled participants over the 24-week treatment period, particularly those for whom long or frequent visits are hardest to manage (for example, because of pain and limited function from active rheumatoid arthritis, work, or caregiving).

Evidence: Cite the specific fact or behavior in the case study or Protocol Summary that supports your concern.

Several participants report that "the repeated visits, injections, blood draws, joint examinations, and questionnaires are burdensome, particularly when visits take longer than expected." The protocol schedules eight visits (Screening, Baseline/Day 1, and Weeks 4, 8, 12, 16, 20, and 24), and Section 9 (Risk 4) recognizes that repeated procedures "may create discomfort, inconvenience, or fatigue."

Why it matters: Explain the potential effect on participant rights, safety, welfare, privacy, or voluntariness.

Burden is a welfare issue in its own right for people already living with pain and limited function. It also affects voluntariness over time: consent is ongoing, and a participant who was not told how demanding visits would be has not had a fair chance to decide. If only people with flexible schedules and easy transport can complete the study, participation becomes inequitable and the results less representative. Missed Week 24 assessments would also weaken the primary endpoint.

Recommended action: State the immediate protection, correction, or escalation you recommend.

Describe the full visit schedule and realistic visit length during consent. Plan each visit so that questionnaires, examination, blood collection, and injection run without avoidable waiting, and collect each sample once. Offer appointment times within the protocol window that suit the participant. Ask about burden at each visit, document what participants say, and report concerns to the PI as Section 9 requires. If a pattern develops, the PI should raise it with the sponsor and ask whether any IRB-approved support is available.

Overall Ethics Assessment

Prompt: Summarize the most important ethical concerns the site should address before or during study conduct and explain why.

The most important concerns are the two rated High alongside the modeled risk, because each can cause direct harm to a participant's rights or safety before anyone notices.

Consent quality comes first. The scenario shows three separate weaknesses before enrollment has properly begun: a staff member promising benefit, a participant ready to sign a form they cannot read, and a discussion shortened to keep the clinic on time. IMMU-201 enrolls people who "remain symptomatic despite stable standard therapy," so hope for relief is strong and the pressure to join is real. The site should treat consent as a protected, unhurried visit, confirm understanding rather than signature, have a documented process for participants who cannot read or whose capacity is uncertain, and never begin a protocol-specific procedure until consent is valid.

Safety reporting must feel safe to participants. IMT-201 alters immune activity, and infection is the first risk named in the protocol. A participant who hides fever and cough to stay in the study is exposed to serious harm. Staff must document and escalate every reported symptom to the investigator promptly, and participants need to hear at consent and at every visit that reporting symptoms protects them and is expected.

Privacy and burden need workflow solutions. Reproductive discussions in an open work area and visits that run long are not isolated lapses; they come from how the clinic is organized. A private room for sensitive procedures and a planned visit flow address both, and they also support honest disclosure, retention, and complete Week 24 data.

A common cause runs through the scenario: time pressure and a wish to keep participants enrolled are being allowed to override protections. Before study start the PI should confirm that every delegated staff member is trained on consent, adverse-event escalation, and confidentiality, and that concerns in these areas are escalated to the PI and, where required, reported to the sponsor and IRB.

Final Self-Check

☑️ I documented at least three distinct ethical risks total, including the modeled risk.

☑️ Each additional risk is supported by a specific fact from the case study or Protocol Summary.

☑️ I identified the population affected by each risk.

☑️ I assigned a severity level and explained the concern.

☑️ Each recommendation is concrete and directly addresses the identified risk.

☑️ My recommendations include escalation when participant rights, safety, or study conduct may be affected.

☑️ I did not simply repeat the modeled risk in different words.

Training note: IMMU-201, the participants, events, and all study information in this template are synthetic and were created solely for this educational activity.